

C808

Classification Systems

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

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How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

Items

1.

A 450-bed teaching hospital that recently upgraded to a new encoder admits a patient who has a laparoscopic appendectomy during the inpatient stay. The surgeon trained at a well-known program. Which code set reports this procedure on the hospital's inpatient facility claim?

- A. HCPCS Level II, since the new encoder maps procedures to it.
- B. ICD-10-CM.
- C. CPT, because the surgeon reports the procedure with CPT.
- D. ICD-10-PCS.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2. [TRANSFER — reversed stem / different variable]

A home health agency needs to bill Medicare for a walker it supplied to a patient. Which code set identifies the walker itself?

- A. ICD-10-CM.
- B. ICD-10-PCS.
- C. CPT Category I.
- D. HCPCS Level II.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3.

A 68-year-old retired teacher who lives with her daughter is admitted through the ED with chest pain. After workup, the physician determines the pain was caused by acute cholecystitis. She also has stable type 2 diabetes. Her insurance is a Medicare Advantage plan. What is the principal diagnosis?

- A. Whichever condition pays most under her Medicare Advantage plan.
- B. Chest pain.
- C. Acute cholecystitis.
- D. Type 2 diabetes.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4.

A patient at a busy urban hospital is discharged after a five-day inpatient stay. The discharge summary, dictated by a hospitalist who covers three units, says “probable bacterial pneumonia,” and the patient was treated with IV antibiotics. How should the coder report the diagnosis?

- A. Query the hospitalist, since stays under a week require confirmation.
- B. Never code any condition labeled “probable.”
- C. Code only the signs and symptoms, such as cough and fever.
- D. Code bacterial pneumonia as if it were confirmed.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5. [TRANSFER — reversed stem / different variable]

In a physician office visit, the provider documents “suspected strep throat” and orders a test; the result is pending when the visit is coded. What is coded?

- A. The sign or symptom, such as sore throat.
- B. Strep pharyngitis, as if confirmed.
- C. A screening Z code for strep.
- D. No diagnosis code until the result returns.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6.

A newly credentialed coder at a multispecialty clinic that sees 200 patients a day looks up a condition in the ICD-10-CM Alphabetic Index and finds a code. The clinic manager wants all charts coded by noon. What must the coder do before assigning the code?

- A. Verify it in the Tabular List, checking instructional notes and any required characters.
- B. Check the Tabular only when the Index shows a “see” note.
- C. Look up the matching diagnosis in the CPT manual.
- D. Assign it directly to meet the noon deadline.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7.

A coder at a pediatric practice with an award-winning asthma program finds two conditions documented. The Tabular entry for the first code has an Excludes1 note listing the second condition, and the provider has not documented them as unrelated. The practice sees many Medicaid patients. What does the note mean?

- A. The two codes should not be reported together.
- B. Both may be coded because both are present.
- C. The note applies only to Medicaid claims.
- D. The second condition must be sequenced first.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8. [TRANSFER — reversed stem / different variable]

A Tabular entry carries an Excludes2 note for a condition the patient also has, documented by the provider. What should the coder do?

- A. Query the provider every time an Excludes2 note appears.
- B. Use a single combination code instead.
- C. Report both codes, since both conditions are present.
- D. Report only one of the two codes.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9.

At a family practice that recently expanded its Saturday hours, a patient comes in for a scheduled mole removal. During the visit the physician also evaluates and manages a new, unrelated problem of uncontrolled blood pressure and documents it separately. The patient has been seen at the practice for nine years. Which modifier is added to the E/M code?

- A. None; an E/M is always bundled into a same-day procedure.
- B. -25
- C. -51
- D. -59

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10.

A community hospital with a new CFO pulls a four-case test sample with MS-DRG relative weights of 1.2, 0.8, 2.5, and 1.5. The hospital's base rate is \$6,000. What is the case mix index for this sample?

- A.** \$9,000
- B.** 1.50
- C.** 6.00
- D.** 2.50

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	____	

11.

A patient who plays in a weekend soccer league is seen in the ED for a wrist fracture, which is set and casted. Two weeks later the patient goes to an orthopedic clinic for a routine cast check; the clinic is new to the patient. Which 7th character applies to the fracture code at the cast-check visit?

- A.** S, sequela.
- B.** A, because the bone is still actively healing.
- C.** D, subsequent encounter with routine healing.
- D.** A, because it is the patient's first visit to this clinic.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	____	

12. [TRANSFER — reversed stem / different variable]

A fracture code on a claim carries 7th character A. What does that tell you about the encounter?

- A.** The encounter is for a late effect of the injury.
- B.** The patient is receiving active treatment for the injury.
- C.** It is the patient's first-ever visit for the injury.
- D.** The patient is in routine healing or aftercare.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	____	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer D Concept 1

A 450-bed teaching hospital ~~that recently upgraded to a new encoder~~ admits a patient who has a laparoscopic appendectomy during the inpatient stay. ~~The surgeon trained at a well-known program.~~ Which code set reports this procedure on the hospital's inpatient facility claim?

Target word: code set for inpatient facility procedures **Decides:** Inpatient hospital (facility) claim.

A	Camouflage	The encoder upgrade does not change which code set applies.
B	Adjacent category	ICD-10-CM reports diagnoses, not procedures.
C	Confusion set	CPT is for the surgeon's professional claim and outpatient services, not the inpatient facility claim.
D	Correct	Hospitals report inpatient procedures with ICD-10-PCS.

2. Answer D Concept 1 • transfer

A home health agency needs to bill Medicare for a walker it supplied to a patient. Which code set identifies the walker itself?

Target word: code set for a supply **Decides:** Durable medical equipment is a HCPCS Level II item.

A	Inverted operation	ICD-10-CM reports why the walker is needed (the diagnosis), not what was supplied.
B	Confusion set	PCS is for inpatient hospital procedures.
C	Adjacent category	CPT Category I describes procedures and services, not equipment.
D	Correct	HCPCS Level II covers durable medical equipment and supplies.

3. Answer C Concept 2

A 68-year-old retired teacher ~~who lives with her daughter~~ is admitted through the ED with chest pain. After workup, the physician determines the pain was caused by acute cholecystitis. She also has stable type 2 diabetes. ~~Her insurance is a Medicare Advantage plan.~~ What is the principal diagnosis?

Target word: principal diagnosis **Decides:** After study, acute cholecystitis caused the admission.

A	Camouflage	Payer and payment never decide the principal diagnosis.
B	Dropped element	Drops "after study"; a symptom is not principal once its cause is found.
C	Correct	It is the condition established after study to be chiefly responsible for the admission.
D	Adjacent category	A coexisting chronic condition is a secondary diagnosis.

4. Answer D Concept 3

A patient at a ~~busy urban~~ hospital is discharged after a five-day inpatient stay. The discharge summary, ~~dictated by a hospitalist who covers three units,~~ says “probable bacterial pneumonia,” and the patient was treated with IV antibiotics. How should the coder report the diagnosis?

Target word: uncertain diagnosis, inpatient **Decides:** Inpatient setting; uncertain diagnosis documented at discharge.

A	Wrong stem number	Length of stay has nothing to do with the uncertain-diagnosis rule.
B	Absolute language	The inpatient guideline specifically allows it.
C	Confusion set	That is the outpatient rule.
D	Correct	Inpatient guidelines code probable conditions documented at discharge as if established.

5. Answer A Concept 3 • transfer

In a physician office visit, the provider documents “suspected strep throat” and orders a test; the result is pending when the visit is coded. What is coded?

Target word: uncertain diagnosis, outpatient **Decides:** Outpatient setting.

A	Correct	Outpatient guidelines do not code uncertain diagnoses; code to the highest certainty, the symptom.
B	Inverted operation	Applies the inpatient rule to an outpatient encounter.
C	Adjacent category	The patient has symptoms, so this is a diagnostic visit, not screening.
D	Absolute language	Every claim needs a diagnosis; the symptom is coded now.

6. Answer A Concept 4

A newly credentialed coder at a multispecialty clinic ~~that sees 200 patients a day~~ looks up a condition in the ICD-10-CM Alphabetic Index and finds a code. ~~The clinic manager wants all charts coded by noon.~~ What must the coder do before assigning the code?

Target word: required coding step **Decides:** The code came only from the Index.

A	Correct	Guidelines require confirming every Index code in the Tabular List.
B	Dropped element	The Tabular check is required every time, not only for cross-references.
C	Confusion set	CPT does not contain diagnosis codes.
D	Camouflage	Reacts to the time pressure; skipping the Tabular risks wrong or incomplete codes.

7. Answer A Concept 5

A coder at a pediatric practice ~~with an award-winning asthma program~~ finds two conditions documented. The Tabular entry for the first code has an Excludes1 note listing the second condition, and the provider has not documented them as unrelated. ~~The practice sees many Medicaid patients.~~ What does the note mean?

Target word: meaning of Excludes1 **Decides:** Excludes1 = mutually exclusive.

A	Correct	Excludes1 marks conditions that cannot occur together as coded.
B	Confusion set	That is the Excludes2 rule.
C	Camouflage	Tabular notes apply regardless of payer.
D	Adjacent category	Sequencing comes from “code first” notes, not Excludes notes.

8. Answer C Concept 5 • transfer

A Tabular entry carries an Excludes2 note for a condition the patient also has, documented by the provider. What should the coder do?

Target word: meaning of Excludes2 **Decides:** Excludes2 = not included here; both may be coded.

A	Absolute language	No query is needed when both conditions are clearly documented.
B	Adjacent category	Combination codes apply only when the classification provides one; the note does not create one.
C	Correct	Excludes2 means the other condition is not part of this code, so both may be reported.
D	Inverted operation	Applies the Excludes1 rule.

9. Answer B Concept 6

At a family practice ~~that recently expanded its Saturday hours~~, a patient comes in for a scheduled mole removal. During the visit the physician also evaluates and manages a new, unrelated problem of uncontrolled blood pressure and documents it separately. ~~The patient has been seen at the practice for nine years.~~ Which modifier is added to the E/M code?

Target word: modifier for same-day E/M **Decides:** Significant, separately identifiable E/M on the day of a procedure.

A	Absolute language	A separately identifiable E/M can be reported with -25.
B	Correct	Modifier -25 identifies a significant, separately identifiable E/M on the same day as a procedure.
C	Adjacent category	-51 marks multiple procedures.
D	Confusion set	-59 marks distinct procedures, not E/M services.

10. Answer B Concept 7

A community hospital ~~with a new CFO~~ pulls a four-case test sample with MS-DRG relative weights of 1.2, 0.8, 2.5, and 1.5. ~~The hospital's base rate is \$6,000.~~ What is the case mix index for this sample?

Target word: CMI **Decides:** Sum of weights 6.0 over 4 cases.

A	Camouflage	Multiplies the CMI by the base rate; that is an average payment, not the CMI.
B	Correct	$(1.2 + 0.8 + 2.5 + 1.5) / 4 = 6.0 / 4 = 1.50$.
C	Dropped element	The sum of weights, not divided by the number of cases.
D	Wrong stem number	The highest single weight.

11. Answer C Concept 8

A patient ~~who plays in a weekend soccer league~~ is seen in the ED for a wrist fracture, which is set and casted. Two weeks later the patient goes to an orthopedic clinic for a routine cast check; ~~the clinic is new to the patient.~~ Which 7th character applies to the fracture code at the cast-check visit?

Target word: 7th character **Decides:** Routine follow-up during healing, no active treatment.

A	Adjacent category	Sequela is a late effect after the injury has healed.
B	Confusion set	Healing is not active treatment; routine healing is D.
C	Correct	A cast check during healing is a subsequent encounter.
D	Camouflage	A means active treatment, not a first visit to a provider.

12. Answer B Concept 8 · transfer

A fracture code on a claim carries 7th character A. What does that tell you about the encounter?

Target word: meaning of A — reversed **Decides:** A = active treatment.

A	Adjacent category	That is S.
B	Correct	A applies whenever active treatment is given, even across multiple visits or providers.
C	Confusion set	A is defined by active treatment, not by visit count.
D	Inverted operation	That is D.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	3	
Wrong stem number	A figure copied from the prompt that is not the answer.	2	
Adjacent category	The right idea in the wrong bucket.	9	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	4	
Absolute language	Always, never, guarantee — where judgment applies.	4	
Confusion set	Every option is real; the test is exact role-matching.	8	
Camouflage	Answers a stem fact that was never the question.	6	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	Which code set reports what	ICD-10-CM reports diagnoses in all settings; ICD-10-PCS reports inpatient hospital procedures; CPT reports physician and outpatient services; HCPCS Level II reports supplies, equipment, drugs, and some services.	Using CPT for the hospital's inpatient procedures, or a diagnosis code for an item supplied.	The setting (inpatient facility vs. outpatient/professional) and whether the thing is a diagnosis, procedure, or supply.
2	Principal diagnosis (inpatient)	The principal diagnosis is the condition established after study to be chiefly responsible for the admission.	Choosing the presenting symptom or the most serious or best-paying condition.	A symptom at admission and a cause found after workup.
3	Uncertain diagnoses by setting	Inpatient: code "probable," "suspected," or "rule out" conditions documented at discharge as if confirmed. Outpatient: do not; code the signs and symptoms.	Applying the inpatient rule in an office visit, or the outpatient rule on an inpatient record.	Qualifier words plus the care setting.
4	Index, then Tabular	Locate a term in the Alphabetic Index, then always verify the code in the Tabular List for notes and required characters.	Assigning a code straight from the Index.	A coder has "found a code" in the Index.
5	Excludes1 vs. Excludes2	Excludes1 means the two conditions are never coded together (mutually exclusive); Excludes2 means "not included here" — code both if both are present.	Treating Excludes2 as a ban on coding both.	The number after "Excludes."
6	CPT modifier -25 vs. -59	Modifier -25 marks a significant, separately identifiable E/M service on the same day as a procedure; -59 marks a distinct procedural service.	Putting -59 on an E/M code.	An office visit plus a procedure on the same day.
7	Case mix index	CMI = sum of MS-DRG relative weights / number of discharges.	Reporting the sum of weights or multiplying by the base rate.	A list of relative weights and a count of cases.
8	ICD-10-CM injury 7th characters	A = active treatment; D = subsequent encounter during routine healing or recovery; S = sequela.	Reading A as "first visit" and D as "any later visit."	Whether the patient is receiving active treatment or routine follow-up care.

Course scope (public description): Classification Systems provides a comprehensive approach to learning about medical coding classification, coding audits, and quality standards. Candidates will be exposed to electronic health record systems and leadership principles as they relate to management of ICD and CPT codes. There are no prerequisites for this course.